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Prevention of Suicide: A Challenge for Community Science

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Although this chapter on suicide prevention is part of a section of this volume entitled "The Prevention of Disorder and the Promotion of Effectiveness," it should be noted at the outset that prenecessary suicidal deaths will not be viewed as evidences of a "disorder," but rather as manifestations of various sociopsychological blights and existential distortions. Further, the converse of suicide prevention is not related to "effectiveness" but rather to the promotion of a self-exulting existence, specifically to the elevative use of life's time. All this is to say that suicidal phenomena are not simply to be understood as disease or disorder, and, at the other and positive reach, the most overarching goal is not simply to lift depressions or to avoid benighted feelings but rather to permit the developmental fulfillment and complete individuation of each person in every community.

CONCEPTS OF DEATH AND SUICIDE

Neither suicide nor death has been adequately conceptualized, even though current writings on these topics are certainly not lacking. Among the recent works of more than ordinary interest one can cite especially the following: Choron (1963, 1964) has performed a signal service by tracing Western man's ideas of death over the past several centuries and by focusing and synthesizing the notions of death in our own time. Feifel (1959) and Fulton (1965) have provided us with separate symposia of harmonic and disparate voices on the topics of death and dying; and, perhaps most important for our current needs, Murray (1967) has invited our attention to the concept of partial death, including death of the inner self and death of the outer or social self; Weisman and Kastenbaum (1967) have explicated one of our own favorite notions (see Litman, Curphey, Shneidman, Farberow, & Tabacknick, 1963), namely, the Psychological Autopsy; and Shneidman as editor (1967) and as author (1963) has tried to add an additional symposium on the broader topic of self-destruction in man and to focus on the role of the individual in his own demise, especially in the large majority of subintentioned deaths. All these are to mention but a few of the current writings in this general area. In sum, there is a lively interest in death.

And what of suicide? Suicide is, by definition, a certain kind of death and, as such, obviously relates to other kinds of death. Suicide has been defined as "the human act of self-inflicted, self-intentioned cessation" (Shneidman, 1968b). Suicide relates to motivation and is intentioned (as opposed to death due to trauma from without, apsychological failure from within, or assault from others); it is total cessation (as opposed to partial deaths or temporary interruption of consciousness); it is individual (as opposed to the decimation or disappearance of a group); it is technically "suicide" (as opposed to Natural, Accident, and Homicide in the NASH (Natural-Accident-Suicide-Homicide) classification of modes of death used for reporting purposes in the death certificates (and statistics) in the Western World;[1] and, in practically every ordinary case, it appears, from the point of view of the surviving

relatives, to be stigmatizing (as opposed to honorific, uplifting, ennobling, comforting).

The extent of the community and mental health problems created by or associated with suicidal deaths is difficult, perhaps impossible to enumerate. The National Center for Health Statistics (1967) estimates over 20,000 suicidal deaths in the United States each year. Some other experts on this topic, notably Dublin (1963), believe that this is a minimum figure, representing a significant underreporting. My own belief is that the veridical data are half again as high, well over 30 thousand self-inflicted deaths per year—and if subintentioned deaths were added, the number would be ten times as high. Further, there are about eight suicide attempts for each reported suicidal death (Farberow & Shneidman, 1961) so that conservatively we are dealing with over 180,000 suicidal episodes each year. The number of people alive in the United States who have attempted suicide at some time in their lives is estimated to be in the millions (Dublin, 1963). Although it seems obvious to say that the primary goal of any suicide prevention effort is to save lives (i.e., to effect a reduction in the suicide rate), it is no contradiction to state that, given the suicide figure at any given time, by far the more important mental health problem relates to the fate and well-being of the survivor-victims of the suicidal deaths. If we can assert that the typical suicide directly affects four people (a surviving parent, a spouse, and two children)—not to count the larger number he less directly touches—then we are talking about the dire and inimical mental health sequelae imposed (usually for the lifetime of that person) on around 100,000 additional persons each year, burdens of guilt, shame, puzzlement, taint, fear, and mystery which are never satisfactorily resolved and often reach through the generations, benighting many lives.

CENTERS FOR SUICIDE PREVENTION

Currently, there is a great deal of activity related to suicide prevention in this country. One of the most impactful developments on the national scene was the establishment, in 1966, of a Center for Studies of Suicide Prevention within the National Institute of Mental Health. The first announcement of this Center was made by Dr. Stanley F. Yolles, Director of the NIMH, at a Symposium on Suicide at the George Washington University School of Medicine in October 1965. At that time, Dr. Yolles (1967, pp. 15–26) indicated that "the Center will have five basic functions in support of the development of this nation's capability to prevent suicide," which he listed as follows:

1. It will serve as the focal point within the Institute to coordinate and direct Institute activities in support of research, pilot studies, training information, and consultation designed to further basic knowledge of the problem of suicide and techniques for aiding the suicidal individual. Studies to be supported will include continued basic and applied research in cultural, sex and age differences in suicide and the relationship of suicidal acts to behavioral disturbances, social and medical problems. Among the latter are homicide, personality disturbances, fatal accidents, occupational and educational stress, terminal illness, use of drugs and alcohol, cultural deprivation, and other aspects of several social stresses. In addition to studies of environmental stress, the Institute will continue to support research into the possible relationship of biochemical processes to suicide. . . .

2. The Center will also compile and disseminate information and training material designed to assist mental health personnel, clergy, police, educators and others in obtaining a better understanding of suicidal actions and learning to utilize research findings.

3. It will assist in developing and experimenting with a variety of regional and local programs and organizational models to coordinate emergency services and techniques of prevention, case finding, treatment, training, and research.

4. The Center will maintain liaison with studies and programs on suicide prevention undertaken by other Federal agencies . . . and with appropriate national and international groups. . . .

5. The Center will promote and maintain the application of research findings by state and local mental health agencies.

On that historic occasion, Dr. Yolles appropriately implied that the intellectual permissiveness for the creation of a national suicide prevention center had been given, in large part, by the successful demonstration of research, training, and clinical effectiveness of the Los Angeles Suicide Prevention Center, started by two psychologists in 1958, after a decade of research and publication, itself supported by an NIMH demonstration grant.

There had, of course, been organized suicide prevention activities in this country since 1906, the year the Save-A-Life-League was founded in New York City, but the Los Angeles Center was apparently the first multidisciplinary Center within the tradition of a scientifically-oriented training and research clinical unit. Within the past decade, four edited books—*Clues to Suicide* (Shneidman & Farberow, 1957), *The Cry for Help* (Farberow & Shneidman, 1961), *Taboo Topics* (Farberow, 1963), *Essays in Self-Destruction* (Shneidman, 1967), and *Psychology of Suicide* (Shneidman, Farberow, & Litman, 1971)—and over one hundred professional articles have come from the Center, in addition to the numerous training workshops, institutes, seminars, consultations, and conferences (Shneidman & Farberow, 1968)—not to mention the substantial number of lives-in-balance believed (by staff and by citizen-caller) to have been saved.

In a recent "progress report" (Shneidman & Farberow, 1965) the cofounders of the Los Angeles Center discussed their own work in terms of its having been a demonstration of public health feasibilities:

> If we have been a demonstration project, what have we demonstrated? Primarily we believe that we have demonstrated certain important *feasibilities*. Webster's dictionary defines "feasible" as "capable of being done or carried out; practicable, possible; reasonable or capable of being used or dealt with successfully." . . . We propose that our experiences at the Suicide Prevention Center in Los Angeles over the last several years have demonstrated the *feasibility of*: (a) Preventing suicide; (b) Discovering prodromal clues to suicide; (c) Doing social research on this topic; (d) Using active therapeutic techniques, often involving the "significant other"; (e) Acting as a consultation service for established health agencies; (f) Working with a chief medical examiner-coroner, especially by use of the "Psychological Autopsy" procedure; (g) Having an around-the-clock service; (h) Employing a truly multi-professional approach; (i) Reconceptualizing some time-worn (and inadequate) concepts of suicide and death; (j) "Un-booing" some unnecessary taboos; (k) Showing the desirability of establishing regional training centers, and (l) Operating a specifically focussed suicide prevention center.

In the past decade, there has been a spirited growth of suicide prevention centers throughout this country. The figures detailing this trend are themselves interesting. As recently as 1958 there were three more-or-less comprehensive suicide prevention centers in this country; in 1959, there were four; in 1960, five; in 1964, there were nine; in 1965, there were 15; in 1966, 30; in 1967, 40; and in 1968 there were 60; in 1972, over 200. Not all of the centers of the future will be autonomous and have separate identification; indeed, most of them will be—as they ought to be—integral aspects of hospitals, universities, and especially of Comprehensive Mental Health Centers, but, nonetheless, they will exist. Of the over 250 Community Mental Health Centers already supported (by staffing or construction grants) by NIMH, there are about 20 in which suicide prevention services and activities were indicated as integral aspects of their initial plans. In terms of geographic distribution, suicide prevention centers now exist in about half the states—21 in 1968—and the District of Columbia—in every major section of the nation. But, obviously, with less than half the total number of states represented, there is much to be done.

A suicide prevention center provides an example par excellence of a kind of service that, literally in order to stay alive (much less to function with any degree of effectiveness), needs to coordinate closely and well with a large number of agencies and key persons within the community. Perhaps more than most, the prevention of suicide is a *community* mental health operation. Experience teaches us that the establishment of suicide prevention facilities within a community is an experience in liaison and coordination. The interest (or, at the least, the passive approval) of several pivotal groups should, in most cases, be secured: the local medical group, the police, local government, hospitals, resource therapists in private practice, some civic groups, a number of social agencies, and the press. A recent Public Affairs Pamphlet (Shneidman & Mandelkorn, 1967) contained the following advice:

> A suicide prevention center cannot open shop all at once like a supermarket. Rather, the entire process, if it would be successful, must be gradually and tactfully woven into the community. From the beginning, the organizers must solicit help—at the very least, cooperation—from the city or county medical authorities. The hospitals, the coroner's office, and the police chief

should know about the beginning of any suicide prevention service. In fact, suicide prevention needs their help. This is reasonable, since the new service ultimately will ease police and hospital emergency-room workloads. But, on occasion, the suicide prevention service will have to call on them for help. The local press, radio, and television should be informed about what's afoot and asked to cooperate. If a story breaks before the budding suicide prevention service is ready, this premature news could be disastrous. Of course, the city government must know what plans are being made. If city officials are not the initial sponsors of such a community service, certainly their endorsement should be heartily pursued. Without local cooperation, successful suicide prevention is practically impossible.

The recent establishment of a new multiprofessional discipline, *suicidology*, serves as the intellectual catchment area for a wide variety of *scientists* (epidemiologists, demographers, statisticians, sociologists, social psychologists, etc.), *clinicians* (psychiatrists, clinical psychologists, psychiatric social workers, trained volunteers, clergy, police, etc.), and *educators* (school and university personnel, health educators, etc.). Suicidology is the study of, and concern with, suicidal phenomena and their prevention. This term was chosen advertantly in order to give the special visibility and identity which such a new discipline required (Shneidman, 1964). As part of the activities of the new Center for Studies of Suicide Prevention at the National Institute of Mental Health, we had sought to create some sense of special excitement in the burgeoning fields of suicide and suicide prevention and to unite the interests of a number of kinds of people concerned either with suicidal phenomena or with suicide prevention. Suicidology seemed to provide a reasonable solution.

In March 1968, the American Association of Suicidology was organized in Chicago, just prior to the annual meetings of the multiprofessional American Orthopsychiatric Association. On that occasion, a number of eminent suicidologists—Jacques Choron, Louis Dublin, Paul Friedman, Robert Havighurst, Lawrence Kubie, Erich Lindemann, Karl Menninger, and Erwin Stengel—"reconvened" the famous 1910 Viennese symposium "On Suicide." The edited proceedings of this meeting are presented in a recent publication (Shneidman, 1969b). A new quarterly journal *Life Threatening Behavior* has been established.

In this chapter, suicide has been viewed as a sociopsychological blight. I am one who believes that psychologists (and other social and behavioral scientists) would do well to focus, even to specialize, in the *substantive* areas of specific sociopsychological blights—suicide, homosexuality, urban perturbation, to name a few. The psychologists's stake—indeed, his responsibility—in suicide and suicide prevention is a vital one. In this, of course, he joins with his colleagues in sociology, psychiatry, public health, philosophy, health education, to name a few. The role of the psychologist in suicidology permits him to do exciting things: research, training, and clinical functions. At this moment, the opportunities in suicide prevention for psychologists in public service would seem to be limited only by the resources of their own imaginations and energies.

A TEMPORAL CONCEPTUALIZATION FOR SUICIDAL PHENOMENA

It is possible that a useful understanding of the prevention of suicide as related to community and mental health issues might be achieved in terms of any one of a number of models or paradigms. At the outset, however, it can be argued that among the models to be chosen, the medical model (with its notions of disease, cause, cure, and a specially anointed curer) seems especially limited and inappropriate. In this connection, Dr. Bertram Brown, Director (then Deputy Director) of the NIHM (and a contributor to this volume) and Dr. Eugene Long (1967) have appraised the situation realistically in their discussion of "The Medical Muddle," a paper presented at the 1967 annual convention of the American Psychological Association.

How to conceptualize an approach to suicidal phenomena within the context of community practice? A rather simple scheme along a temporal dimension is proposed which divides all activities in terms of whether they occur before, during, or after the suicidal crisis. From this (temporal) point of view—using the Latin word *venire* meaning "to come"—one can come (or do) before (*prevention*); one can come (or do) during or between (*intervention*); and one can come (or do) after (*postvention*). These three temporal points exhaust the possible times at which action—community or otherwise—can be effected. They focus on

the *whens*, rather than the *whos* (M.D., Ph.D., D.D.), the *wheres* (*inpatient*, *outpatient*, at home, on the telephone), or the *whats* (manic depressive-depressed, paranoid schizophrenic, benighted citizen, perturbed housewife, etc.). This way of "seeing the action" has the additional advantage of "opening the field" to anyone who can help at any crucial point in time: a trained volunteer, a significant (or even insignificant) other, a relatively untrained psychiatrist, an epidemiologist, a demographer, a psychoanalyst, a vital statistician, a clergyman, a sociologist, a policeman, a social psychologist, a health educator, a clinical psychologist—a whole panoply of individuals who can practice prevention, intervention, or postvention, as clinicians, research scientists, or as emphathic human beings seeking to help someone in duress (Shneidman, 1971).

A simple pictorial representation in this scheme might be contained in a grid, in which the major threads of this proposal—prevention, intervention, and postvention—would be represented by the rows, and the major functions of any enterprise—clinical services, training and education, research and investigation, and administration and liaison—might be represented by the columns. Within this potentially fruitful nexus, any suicidologist ought to be able to find a useful area for his own special talents—the direction, leadership, or supervision of any professional person who is most suited to these latter tasks.

In general, there are three main avenues to the reduction of suicidal deaths which emerge from their temporal framework: 1) to increase the acumen for recognition of potential suicide among all possible rescuers; 2) to facilitate the ease with which each citizen can utter a cry for help; and 3) to provide resources for managing suicididal crises.

The key to the reduction of suicide lies in recognition and diagnosis—the perception of the premonitory signs and the prodromal clues. Most individuals who are suicidal typically cast some verbal or behavioral shadows before them. Prevention lies in recognition. This task of early case-finding must be shared by both professionals and lay people. The "early signs" of suicide must be made known to each physician, clergyman, policeman, and educator in the land—and to each spouse, parent, neighbor, and friend. There is a giant body of mythology and erroneous folklore concerning suicide. One of the first tasks is to disseminate the solidly known facts to all possible citizens.

The tabooed nature of suicide must be recognized. Part of a successful program of suicide prevention lies in reducing the taboos and giving a greater permissiveness for citizens-in-distress to seek help and to make their plight a legitimate reason for treatment and assistance.

Both facilities and personnel will be needed. The personnel will need to acquire relevant skills and appropriate attitudes. Management and treatment of the suicidal individual, as well as his "significant others"—within their own cultural setting—will be required for reduction of suicide rate.

The remainder of this chapter will attempt an explication of these notions of prevention, intervention, and postvention. Under each of these three basic categories—prevention, intervention, and postvention—an attempt is made to develop a number of key foci for action, which, when taken together, should constitute a somewhat comprehensive community program for suicide prevention.

PREVENTION

In the context of suicide prevention has the general meaning of "to forestall," to make unnecessary, to preclude, avert, or to ward off. In common usage, it means not only to prevent the occurrence of a suicidal condition or state of mind. In a much broader sense—in terms of long-range policy planning, perhaps—prevention planning would have to include two major goals, which might seem, at one level, to be contradictory ones: (a) the goal of reducing the number of suicidal deaths (and the simultaneous unequivocal demonstration that the reduction claimed had, indeed, been effected); and (b) the reduction in the stigma or taboo on suicide, so that even if it did occur the mental health sequelae would be less severe and less crippling These two major goals would, it seems to me, also be applicable in other mental health areas, such as schizophrenia or homosexuality.

With these notions in mind, the following are suggested as parts of a comprehensive program in suicide prevention, especially relating to the preventive aspects of such a program.

A Redefinition and Refinement of Concepts and Statistics on Suicide

It would seem to be part of the elementary logic of a clinical science to believe that remediation optimally follows from one's understanding of etiology and that both of these follow from one's understanding of the nature of the phenomena. Good conceptualizations (including definition and taxonomy) must precede any effective action (prevention, intervention, or postvention).

It is generally agreed that current statistics on suicide are grossly inadequate and that comparisons of suicidal incidents between cities, between states, and between countries, based on available figures, are at best sometimes inaccurate and often obfuscatory and misleading. The current inaccuracies are due to many reasons, including the following:

(a) Confusion as to how to certify equivocal deaths, for example, those which lie between suicide and accident;

(b) Dissembling on the part of police and physicians who wish to protect the family, and public officials who wish to protect the reputation of their community;

(c) Inaccurate record-keeping, where what could be known and ascertained simply is not accurately tabulated;

(d) The irremediable inadequacies of the present concepts.

More will be said of this last point in the paragraphs below.

Two additional points should be made in relation to statistics: An opportunity now exists to introduce improved classifications and to conduct pilot studies to determine (for the first time) the veridical suicide rates in some selected communities; and further, the specter exists that unless there is a refinement of the concepts related to self-destruction, there will never be accuracy of reporting, because the present concepts are simply not conceptually strong enough to reflect accurately the events which they are purported to represent.

At all levels, special thought should be given to the redefinition and refinement of statistics and to the problem of record-keeping to the end of suggesting new concepts and a comprehensive program for uniform national record-keeping in relation to self-destruction. We have already suggested that the greatest difficulty in accurate reporting of self-destructive deaths is that it is currently tied to the archaic NASH classification. The greatest shortcoming of this classification is that it completely omits the role of the individual in his own demise.

The traditional NASH classification of death robs us of the possibility of generating meaningful statistics. The approach suggested above, focusing on the *intention* of the individual, if used in conjunction with the traditional approach, might well provide an important step in the psychological understanding of a broad spectrum of deaths and lead to more effective assessment and prevention.

Special Programs for the "Gatekeepers" of Suicide Prevention

An important key to suicide prevention lies in detection and diagnosis. One of the most important findings from the last decade's experience in suicide prevention is that practically every person who kills himself gives some verbal or behavioral clues of his intention to do so. These prodromal clues are often in "code," are cryptic or disguised, but nonetheless they are clues, and one can learn to recognize them. These are the "handles" to prevention.

In practice, a variety of kinds of people hear the presuicidal clues—spouse, friend, neighbor, clergyman, policeman, bartender, physician, employer, etc.; however, it is a most important fact that over 65 percent of all individuals who commit suicide have seen a *physician* (usually a general practitioner) within three months of the event. It is therefore crucial to have a program of education relating to detection and diagnosis which focuses on physicians, and secondly on clergy, police, and other "gatekeepers."

There are a number of ways in which a program for educating general practitioners about the diagnostic indices of suicide can be done. These include the following:

(a) Preparation of special educational materials for physicians focused on the premonitory signs of suicide. These materials can be in the form of brochures, pamphlets, long-play training records, film strips, films, etc.;

(b) A similar program directed toward physicians in hospitals and clinics;

(c) Including instruction on suicide prevention for medical school students by introducing such materials into the medical school curriculum;

(d) Courses on suicide prevention in postgraduate medical education in medical schools throughout the country;

(e) The use of resources of the American Academy of General Practitioners (AAGP);

(f) Special national and regional conferences on suicide prevention sponsored by the AMA and the AAGP, perhaps with cosponsorship by the American Psychological Association and the National Association of Social Workers;

(g) Suggesting that the drug companies train their drug detail men in suicidal prevention. Although every doctor in the United States does not read every journal or attend conventions, every doctor does see drug detail men. The major drug companies ought to be most willing to train these drug detail men in the principles and content of suicide prevention and to have them distribute appropriate literature to the physicians on whom they call. This could be done not only through the drug companies, but with the additional coordination of the AMA, state medical societies, etc.

Clinical experience, now over the past several years at the Los Angeles Suicide Prevention Center, indicates that it is not only possible but advisable for physicians to ask direct questions about a patient's suicidal intent without any harmful effects. In interviews with 10,000 suicidal patients and reviews of 3,000 suicidal deaths, the staff at the Los Angeles Center found no evidence that such questions had ever harmed patients; indeed, the questions often relieved the patients and permitted them to discuss their own problems with their physicians.

Additional special programs, tailor-made for clergy, police, educators, and others, should, without question, also be considered.

Carefully Prepared Programs in Massive Public Education

This is probably the most important single item for effective suicide prevention and at the same time one of the most difficult to put into practice in ways which would be both acceptable and effective. The basic notion is that one major avenue to reduction of suicidal deaths is through the use of the lay citizen as one for first-line detection and diagnosis. The rough model may be found in cancer detection, wherein more and more citizens know the prodromal clues for cancer (e.g., bleeding from an aperture, a lump in the breast, a wart or sore that does not heal or which grows, etc.). The same model, with appropriate changes, might well be adopted in suicide prevention.

A study of massive public education might be done initially in a few carefully preselected communities. This study would need to involve experts in epidemiology, biostatistics, and the use of communication media, especially. This type of study would need to be preceded by a long-term comprehensive study of the actual state of suicidal (and suicidal equivalent) incidents in those areas. The public education activities might include planned and careful use of all the public media: schools, TV, newspapers, radio, advertisements, placards—and these might be done in both usual and unusual places, where appropriate, such as doctors' offices, pool halls, public lavatories, etc.

One or two cities might be selected as large scale pilot projects (or one or two sections of cities might be selected with equal catchment areas). There is the need for control scientific data in order to ascertain the effects and effectiveness of such a program of public education. In part, this can be done by selecting other cities (or sections of cities) which are comparable to the experimental cities in terms of the major variables thought to be relevant.

The use of certain carefully selected target cities as a pilot attempt in suicide prevention is in line with current scientific practice. There currently exist many concepts in suicide prevention which have been found useful: there is already a base of knowledge from which to predict a hope of success.

Although a program of massive public education would seem to be very important in any full-scale assault on the problem of suicide, it would need to be done with the reservations concerning the unanticipated consequences of public information and of popularizing the topic of suicide prevention. We do not know enough about what the short-term effects of such a program might be. However, it needs to be further stated that this in itself is a legitimate subject for serious study and

one in which sociology consultants would play an important role.

The Development of a Cadre of Trained, Dedicated Professionals

The fact is that there do not exist at present trained professionals in sufficient number to man the proposed and projected projects in suicide prevention. There is an acute need for the creation of a core group of individuals who might then direct and staff the suicide prevention programs in the communities throughout the country. It should be pointed out that what is being proposed here is not the training of individuals to be specialists in suicide in the sense of only being therapists for suicidal people, but, rather, that individuals be given sufficient training in the basic ideas and facts about suicide and suicide prevention so that they can then act more meaningfully in their administrative and technical capacities.

This aspect of the total suicide prevention program has already been implemented by the establishment of Fellowships in Suicidology at The Johns Hopkins University. The goal is to have similar programs at main centers of learning at several strategic places throughout the country. Already, it appears that there may be Fellowships in Suicidology programs soon on the West Coast, the Midwest, and the Southwest. The concept seems viable and the future seems to be one of expansion of such training sites.

The main point to be made is that the establishment of these multiprofessional Fellowships in Suicidology will, within a relatively few years, create a corps of trained professionals specifically concerned with suicide prevention and able to staff effectively a variety of suicide prevention activities throughout the country.

INTERVENTION

Most suicide prevention activities are interventative in nature. In part, this is so because this is where the light is, but this fact does not gainsay another fact that great needs, especially for more fundamental research, lie in the areas of prevention and postvention. Intervention has the apparent attractions of immediacy, drama, and relatively quick response. The issue has been identified—he *is* suicidal—and the need is real. Intervention is a kind of secondary prevention, having to do with the effective treatment of an existing, identifiable (suicidal) crisis. The increase in the number of suicide prevention centers from three to over 60 in the past decade is only one of several evidences of the legitimate appeal and humane worthwhileness of intervention. It is largely, but not entirely, the clinician's domain; his and the effective volunteer's.

Making Available Service and Treatment and the Improvement of Treatment Procedures

Although there is great need for systematic research on intervention, the heart of intervention is the service itself. The challenge is a logistic and tactical one: that the services which can be rendered be made *available* to whom they are needed when they are required. This routinely means some kind of 24-hour-a-day operation, usually involving the use of the telephone as the life-saving instrument. The use of the telephone willy-nilly changes the clinical interviewing task, presenting fresh challenges and offering new opportunities. Suicide prevention personnel need to become expert in quickly rating each caller's *lethality* (see Shneidman, 1969a)—his probability of *committing* suicide in the immediate future—as opposed to his perturbation or degree of upset or distress. Admittedly, workers at a suicide prevention center will probably handle more nonsuicidal calls (of individuals lonely, perturbed, intoxicated, psychotic, fundless—but not lethal) than suicidal ones, yet they do well constantly to remember that their clinical goal is to keep people out of the coroner's office, to prevent their killing themselves, and that everything else takes its place subsumed under that primary aim.

The improvement of treatment procedures involves the search for new and improved methods. Within the last decade, a number of changes have already taken place. Among these can be listed a more active approach to treatment in general, the dyadic form of most treatment of suicidal persons actively involving the "significant other," and a great movement "out of the office" into the community, using the resources within the community and putting the organized and unorganized helping hands which exist in the community on the handles which can be identified

on the caller. In this special sense, it can be said that the best suicide prevention worker is the one who is able to get others to do most of the life-saving.

Studies of Special Groups

For any one of a number of reasons, the closer scrutiny of certain groups seems to merit special interest and attention. These groups, in the suicide domain, should include such high-risk groups as college students, military personnel, certain professional groups such as physicians, particularly psychiatrists, the aged, and the identification of other high-risk groups, whatever their composition.

On the topic of college students and suicide, there are currently a number of studies throughout the country. The identification of high-risk groups is a way of redefining individuals who manifest the prodromata or premonitory signs associated with suicidal behaviors. The key issue is whether *groups* of such persons can be found or whether this kind of search is not always a quest for individuals.

Selection and Training of Volunteer Groups

The "manpower problem" in suicide efforts cannot be ignored. One way of addressing it is through the Fellowships in Suicidology program, giving focused training to individuals who already possess graduate or professional degrees. Another way is to turn to the much larger resource pool of mature and willing individuals, carefully to select them, and then rigorously to train them. This has been the route of many of the suicide prevention centers through the country, and there are some excellent reports of these experiences (see Heilig, Farberow & Litman, 1968). In general, selection has focused on such traits as flexibility and trainability, absence of "overinvestment," or previous emotional difficulties, considerable maturity and some experience with life's problems—the avoidance of "psychological virgins"—and the ability quickly to master the difference between a conversation (which is social and coequal) and an interview (which is clinical and betokens the helper and the helped). In the training of volunteers, many usual and innovative techniques have been employed, including the use of taped telephone calls, psychodrama, role playing, and the preceptor method—most of which cost very little but yet have the potential of enormous yield in dedicated and high-morale and low-cost personnel—usually the womenpower of the community.

POSTVENTION

We have tried to indicate that the largest mental health problem in relation to suicide relates to the survivor-victims, who outnumber the deceased in the order of five to one. Appropriate care or treatment of a surviving widow, or, especially, surviving young children is obvious good mental health practice (Silverman, 1969). We know a good deal about the nefarious sequelae of a parent's suicidal death and much is known relating to remedial and prophylactic psychological treatment of these unfortunates. And obviously, follow-up programs which study and help individuals who have attempted suicide are needed, in order, at least, to learn more of the "natural history" of suicidal behaviors. Apropos of prophylaxis: postvention of individuals in the present becomes prevention for the next decade, or even for the next generation. Postvention relates to the reduction in the amount of disability in the survivor caused by the irreversible suicidal event, or in the individual subsequent to his own attempted suicide.

A Special Program for Follow-up of Suicide Attempts

We know that about eight out of ten people who commit suicide have previously attempted or threatened it, but the data relating to the percentage of people who have attempted or threatened suicide who subsequently commit suicide are contradictory and equivocal. The primary purpose of follow-up of suicide attempts would be to prevent the commission of suicide. Some people who committ suicide do so the first time they attempt it, but the more common pattern is that of a series of attempts, with increasing lethality, and there are too many reports of individuals who have been sewn up or pumped out and released only to complete the task within hours. Like the suggested programs for the gatekeepers and the program in public education, this also is meant to "nibble" at the

suicide problem and to help effect a reduction in the suicide rate.

A program of follow-up of suicide attempters could be done in sites where the conditions are propitious for success, and with especially cooperating hospitals, police departments, and public health officials. The actual follow-up could be done by a variety of types of personnel, including public health nurses, social workers, psychologists. The data already available particularly from the work of Stengel (1964) would serve as a beginning for further and better understanding.

There is great confusion about the relationship between attempted suicide and committed suicide. (Again, this confusion exists largely because clinicians and investigators fail to think in terms of lethality, as opposed to perturbation. A suicidal event—whether a threat or an attempt or a commission—is best understood in terms of its lethal intention, rather than its method, or how much general upset accompanies it.) We need to know the characteristics of those with low lethality. Obviously, prevention of suicidal deaths lies in dealing with the former.

It might be well to pattern the follow-up procedures for suicide attempts roughly after that of health educators working with VD or TB follow-up, and look forward to the time when suicide attempt follow-up can be built into routine health services. The follow-up could be seen as "posterisis follow-up" and would be a legitimate aspect of a comprehensive approach to suicide prevention. It is known that the most dangerous period with relation to suicide is within three months after a suicidal crisis. A follow-up procedure might be one effective way in saving some lives and would furnish excellent data for significant study and research.

A Special Follow-up Program for the Survivor-Victims of Individuals Who Have Committed Suicide

It is not inaccurate to state that from the point of view of the survivor, there are two kinds of deaths: all the deaths from cancer, heart, accident, etc., on the one hand, and suicidal deaths on the other. If one stops to consider the kind of grief-work and mourning that one has to do on the occasion of a death of a loved one who dies of a natural or accidental cause on the one hand, and then what he has to do for the rest of his life if his parent or spouse has committed suicide, the contrast is then clear. The individual who commits suicide often sentences the survivor to obsess for the rest of his life about the suicidal death. The suicide puts his skeleton in the survivor's psychological closet. No other kind of death in our society creates such lasting emotional scars as does a suicidal death. A comprehensive suicide prevention program should attend to the psychological needs of the stigmatized survivors, especially the children who survive a parent who has committed suicide.

Although this aspect of the program is not directed especially toward reducing suicide—dealing as it does with an individual who has already killed himself—nevertheless, because it relates to the survivors of the suicidal death, it is directly in the center of mental health concern. Today each citizen enjoys many rights in this country; we would hope that he might be granted the right to lead an unstigmatized life, especially a life unstigmatized by the suicidal death of a parent or a spouse.

The Los Angeles Suicide Prevention Center has pioneered in developing a procedure which they have called the Psychological Autopsy. This process is used in cases of equivocal suicidal-accidental deaths, and consists of interviewing a number of individuals who knew the deceased, to obtain pertinent psychological data about the nature of the death. The relevance of this is that it has demonstrated that it is easily possible, always therapeutic, to work with survivors of a suicidal death especially immediately after the death and even for some interval thereafter.

Studies of the effects of suicides on survivors need to be done. Two kinds of studies immediately suggest themselves: retrospective studies of individuals whose parent committed suicide one, five, ten, 20 years ago; and prospective studies, where the suicide has occurred in the very recent past, and the effects on the survivor are followed through time.

We do not at present know of the "cost" of each suicide in terms of the deleterious mental affects on the survivors—how many survivors of a father's or a mother's suicide subsequently need mental hospitalization or other mental health care—and in ascertaining of what these facts are, we need to

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develop special ways for effectively helping individuals who have suffered this kind of traumatic loss. Just as there are better and worse ways of responding to, for example, the loss of a limb or to blindness, so we must develop better ways to help survivors respond to the grim fact of suicide in their family, and thus to reduce the overall mental health toll.

A Rigorous Program for the Evaluation of the Effectiveness of Suicide Prevention Activities

Evaluation is a necessary part of effective follow-up. The goal of effecting a reduction in suicidal deaths carries with it the simultaneous charge of doing so in such a way as to be able to demonstrate unequivocally that those lives have been saved.

Although, in the individual case, suicide can best be seen as reflecting "a damp, dismal November in the soul," it also seems to be so that, in the large, suicide rates vary with such items as the nation's position in relation to peace and war; changes in the economic state of the nation (prosperity or depression); changes, in any one place, in the percentage and the role of Negroes, etc. All this is to say that it is an extremely thorny methodological problem in epidemiology to make a single test or to prove the effectiveness of a suicide prevention program in terms of a single measure, especially suicidal deaths. Nevertheless, efforts to establish the effectiveness of suicide prevention activities must, from both a scientific and moral point of view, be a part of a comprehensive suicide prevention program from its very beginning. Without this feature of rigorous evaluation there can be no accounting by any clinician or investigator, either to himself or to the scientific community.

This aspect of the total program, perhaps more than some of the others, requires close consultation with people in biometry, in epidemiology, in sociological methodology, in research design, and in statistics. Perhaps special committees might be formed specifically to deal with the issue of evaluation.

The basic issues in a suicide prevention program are: What can a local suicide prevention program do which will affect the suicide rate of the people of that community, and how can we find out whether this is being accomplished.

In general, three levels of prevention have been envisaged: Primary prevention, in which the goal is to make it unnecessary for the suicidal crisis ever to occur; secondary prevention (intervention) which has to do with the effective treatment of an existing suicidal crisis; and tertiary prevention (postvention) which relates to the reduction in the amount of disability in the survivor caused by the irreversible (already occurred) suicidal event.

And if there are three levels of prevention, there are at least three criteria for the effectiveness of any suicide prevention program. The first, and most obvious, and by far the most important, is the reduction of suicidal deaths; the second is the evaluation of the effectiveness of various types of approaches to and treatments of suicidal phenomena; but the third should not be entirely ignored: it is the reduction of the over-all "lethality" in the individuals who make up a community. ("Lethality" is the measure of the probability of an individual's effecting his own death—whether naturally, accidentally, suicidally, or homicidally—within the immediate future.) Just as one might ask what a random study of blood samples of individuals entering a business or government building would reveal in terms of barbiturate and ethanol levels, so in the same spirit one might ask what a random study would reveal in relation to individuals' lethality indices, i.e., their general ties to life. A successful suicide prevention program should, in addition to overtly saving lives, also serve to lower the "lethality level" and "suicidal index" of a community. We need much baseline data in this area.

The very establishment of a suicide prevention program has a salutary effect on the mental health within its own community. It can provide a model for the effective approach to a variety of other sociopsychological blights, as well as provide useful information which can help reduce the inimical effects of these blights. It would be hard to conceive that information generated in the area of suicide prevention would not have implications (both methodological and substantive) for accident fatalities, addiction, alcoholism, delinquency, homicide, schizophrenia, and other maladaptive and self-destructive patterns.

Notes

Written while the author was Chief, Center for Studies of Suicide Prevention, National Institute of Mental Health, Chevy Chase, Maryland, 1966–1969.

1. We would do well to abandon completely the NASH classification of deaths—for it is Cartesian and apsychological in that it entirely omits the role of the individual in his own death and totally disregards the teachings of 20th century psychodynamic psychology. Instead we should attempt to conceptualize all human deaths in terms of a motivational dimension of intention-toward-death. As a beginning, three large subcategories are suggested: intentioned, subintentioned, and unintentioned, which might briefly be defined as follows: An *intentioned* death is any death, from whatever cause(s) or of whatever apparent mode, in which the decedent played a direct, conscious role in effecting his own demise; a *subintentioned* death is one in which the decedent played an indirect, covert, partial, conscious, or unconscious role in *hastening* his own demise by such behaviors as imprudence, excessive risk-taking, abuse of alcohol, misuse of drugs, disregard of life-extending medical regimen, death-risking life style, etc.; and an *unintentioned* death is one in which the decedent played no effective role in effecting his own demise, in that the death is due entirely to assault or trauma from without or nonpsychologically tinged failure from within. The reader is referred elsewhere for a further explication of these notions (Shneidman, 1963, 1968a, 1968c).

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